Supplementary File 1 Study characteristics by screening location

First Author, Year	Location	Study setting (Community – Outreach or Agency, Clinical, Hospital, etc.)	Who Conducted / Provided the Screening	Period of Study	Study Population Type	Control or Comparison Group and Effect of Incentives on Screening	Sample Size (n)	What incentive was given for and STBBI tested
Community Based Screening Sites								
Bowden 2012 [25]	Australian Capital Territory, Australia	Community events - 18 events conducted at 10 separate sites: tertiary campuses, health club, football club, and Aboriginal youth centre	Staff from Canberra Sexual Health Centre and Sexual Health Planning Australian Capital Territory	Oct 2009 to Jun 2010	Targeted youth aged 16-30, those older than 30 were not refused screening (range 13 to 63 years)	No control or comparison group	n=3011 individuals provided with written and/or oral information on chlamydia screening and follow up n=1318 Provided a urine sample n=1077 (81.7%) of those tested were within the target age range of 16 to 30 years	Return of a urine sample for Chlamydia screening
Currie 2013 [24]	Australian Capital Territory, Australia	Community – four city and two suburban pharmacies	Pharmacists	31 Jan 2011 to 28 Feb 2011	Sexually active youth aged 16-30, could self refer or be invited by pharmacy staff to take part	Comparison with other Australian Capital Territory data from February 2011 (during time of program) and February 2010 (no incentive) February 2011, conducted 1120 chlamydia tests for people aged 15–30 years, 900 (80%) for the pharmacy study, 120.0% more than during the same period in 2010 (n=509) and 75% more when only urine samples from individual participants are included If the number of tests in February 2011 (n=1120) is compared with the median number of tests per month for 2010 (n=385) the increase is 190.9%	n=979 screening kits were requested n=900 (93%) urine samples returned n=671 of 900 (75%) urine samples were from unique individuals	Return of a urine sample for Chlamydia screening, contact details, and completed brief questionnaire
Denton 2018 [28]	Alabama, USA	Community - probation and parole office waiting room, onsite screening three days per month	Trained student assistants provided education, does not specify who completed screening	2016 Three scheduled reporting days each month	Parolees and probationers (offenders)	No control or comparison group	n=2,276 offenders approached n=569 (25%) agreed to screening	HIV screening

Dolan 2014 [29]	England, United Kingdom	Community - participants request self-collection kits online or through text message at Freetest.me	Self-collected specimens	Round 1: 18 May to 3 June 2011 Round 2: 8-18 May 2012	Youth aged 16-24 requesting test kits	No statistically significant difference in specimen return rates between control group and incentive group, or among different incentive groups	Round 1: n=1489 Kits requested Round 2: n=1500 Kits requested Total n=2988 Test kit return rate 71.0%	Returning by mail Chlamydia specimen kit (urine sample or vaginal swab) within 30 days Two incentive groups were not contingent on returning the kit
Niza 2014 [31s]	London, United Kingdom	Community – four student residence halls from three universities	Self-test	Not specified	Youth aged 18-24 living in student halls of residence	Increased screening by 1.3% (lottery), 21.3% (voucher). Any incentive (8.9% return rate) statistically significant ($p<.001$) compared to no incentive (1.5% return rate) at 95% CI (1.159, 4.279)	n= 1060	Return of a Chlamydia screening kit same or next day
Parker 2015 [23]	Australian Capital Territory, Australia	Community – six community pharmacies	Pharmacy staff	4 weeks duration in 2011 Dates not specified	Pharmacy attendees aged 16-30 years	No control or comparison group	n=979 Chlamydia tests with questionnaire distributed n=945 (96.5%) questionnaires returned n=78 Participants included contact details for follow up telephone interview n=18 semi structured telephone interviews completed	Return urine sample for Chlamydia screening, related documents and questionnaire
Saxena 2016 [32s]	California, USA	Community - Residential drug abuse treatment program for male parolees	Community health organizations mobile screening unit on site once per month	Not specified	Male parolees entering community treatment	Incentive effective when prior risk behaviour and relevant demographic variables controlled for (OR = 1.99, $p<0.5$, 95% CI [1.05, 3.78])	Total n = 202 Education and incentive n = 104 Control, education only n = 98	HIV screening If already HIV positive, other STDs and/or hepatitis
Clinical Based Screening Sites								
Haukoos 2005 [27]	Los Angeles, USA	Hospital - on site HIV clinic, referred from emergency department	HIV clinic	10 Jan 2002 to 1 Sep 2003 6 month time period,	Emergency department patients identified as at increased risk for HIV	Incentive increased screening uptake (OR = 3.4; 95% CI = 1.8 to 6.3)	n=372 patients were referred for HIV counseling and screening control periods:	Completing HIV counselling and screening post referral from emergency department

				preceded and followed by 6 month control periods	infection by emergency physicians using CDC-based guidelines		n=252 referred n= 20 (8%) completed HIV counseling and screening intervention period: n=120 referred n= 27 (23%) of patients completed HIV counseling and screening	
Montoy 2018 [30]	California, USA	Hospital emergency department	Study staff approached eligible participant, clinician added screening to routine ED care	18 June 2011 to 30 June 2013	Patients aged 13–64 receiving care in an urban emergency department	$5 and $10 incentives increased uptake by 10.5 and 15 percentage points, respectively (95% CI 7.5 to 13.4 and 11.8 to 18.1). $1 incentive not found to increase screening	n=10,463 eligible participants approached and offered HIV tests and questionnaires n=8,715 (82.3%) consented to the study n=4,831 (55.4%) accepted HIV screening	Completing Rapid HIV screening
Stevens 2013 [33s]	Queensland, Australia	Clinical sites - three general practices, four sexual health clinics, recruitment at four sex-on-premises venues (SOPVs)	General practice or STI clinic staff	6 month duration Dates not specified	Attendees of Sex-on-premises venues (SOPVs)	No control or comparison group	n=244 cards distributed to patrons of four (SOPV) venues n=10 presented for screening at one of the clinical sites n=3 of these 10 used their free entry passes for admission to attend one of the SOPV venues	HIV, syphilis, chlamydia and gonorrhoea Incentive given for completing STI screening
Wagner 2020 [34s]	San Francisco, USA	Hospital emergency department	Emergency department staff (nurses, doctors)	1- year duration Dates not specified	Emergency department patients aged 13 – 64	Switching from opt-in to opt-out results in 56% increase in diagnoses at $3807 per new diagnosis. Switching from no incentives to $1, $5, or $10 incentive adds 20, 19 and 41% increases in new diagnoses at $11, 050, $17, 984, and $15, 298 per new diagnosis respectively	See Montoy et al. (2018)	HIV screening
Zenner 2012 [35s]	England, United Kingdom	Primary Care Trusts (PCTs)	Primary Care Trusts	Survey: April 2009 Telephone interviews: Sept-Oct 2009	Youth aged 15-24 accessing PCTs that had and had not used incentives	A single variable analysis demonstrated that unadjusted rate changes were more pronounced in PCTs which used incentives (1.08%, CI 0.48% to 1.68%) compared to those that had not (0.41%, CI 0.003% to 0.82%. The average difference between paired PCTs (0.67%, CI 0.1% to 1.24%) was significant	Evaluated n=65 patient financial incentive schemes used by 46 of the 152 Primary Care Trusts across England Prize and voucher schemes n=46	Chlamydia screening

				Evaluation period: 1 January 2007 to 30 June 2009		(paired t-test p = 0.02). The screening rate difference was more pronounced in voucher schemes compared to prize draws (2.35% vs. 0.16%), and in females compared to males (0.73% vs. 0.14%). Further analysis of voucher schemes found significant effects in females (3.18%) and to a lesser extent in males (1.55%).	No incentive n=42 Excluded non monetary schemes n=19 No note of number of individuals tested	
Mixed Screening Sites								
Currie 2010 [26]	Australian Capital Territory, Australia	Tertiary institutions (Phase 1) Off-campus sexual health facility (Phase 2)	Project nurses and health promotion staff	Phase 1: 13 Feb 2007 - 16 Aug 2007 Phase 2: 13-16 August 2007 (4 days)	Tertiary students aged 16-25 years	Phase 1: Non monetary incentive Chlamydia screening rate 22.9% (95% CI: 21.3–24.4) No. (percentage) of chlamydia positive students 11 (1.7%) 95% CI: 0.9–3.1 Phase 2: Monetary incentive Chlamydia screening rate 42.4% (95% CI: 39.1–45.7) No. (percentage) of chlamydia positive students 7 (1.8%) 95% CI: 0.7–3.6	Phase 1: n=2786 students attended the 31 activities n=638 students (22.9%) provided urine specimens for chlamydia screening Phase 2: n=866 text messages sent resulting in urine specimens from n=392 students (45.3%)	Return of a urine sample for Chlamydia screening